

Eczema Condition Full Report & Treatment Guide

1. Your Problem (Diagnosis)

You have chronic eczema with sensitive, dry, inflamed skin. Repeated use of a strong steroid (Clop-M, containing Clobetasol) on the face and eyelids caused steroid-modified skin and rebound eczema. Your patches on fingers, ankles, elbows are consistent with chronic eczema.

2. What You Had Used Earlier

- Clop-M (Clobetasol + Miconazole)
- Luliconazole 20% (on eyelids and face)
- Moisturizer (Duramoist)
- Face wash (Galglow)
- Sunscreen (Uvedge)

Clop-M used repeatedly caused your skin barrier to weaken and become sensitive.

3. What Your Doctor Prescribed

First Visit:

- Clop-M + Luliconazole (short-term use only)
- Antihistamine (Getzine FX)
- Duramoist
- Galglow face wash
- Uvedge sunscreen

Second Visit:

- Stopped all strong steroids
- Only asked to use moisturizer, sunscreen, antihistamine, face wash.

4. What You Have to Do Now (Correct Treatment Plan)

A) Eyelids:

- Hydrocortisone 1% once nightly for 5–7 days
- Moisturize twice daily (Cetaphil/CeraVe/Vaseline)
- Stop Clop-M and Luliconazole on eyelids

B) Ankles, Elbows, Fingers:

- Mometasone 0.1% once daily for 7–10 days

- Then stop and moisturize 2–3 times/day

C) Face:

- Use Galglow/Cetaphil gentle wash
- Sunscreen daily
- Moisturizer at night

D) Bath Routine:

- Lukewarm water only
- Mild soaps (Cetaphil/Dove sensitive)
- Moisturize within 2 minutes after bath

5. What To Do If It Comes Back in the Future (Flare-Up Plan)

Mild flare:

- Hydrocortisone 1% for 3–5 days

Moderate flare (body):

- Mometasone for 5–7 days

Never use again:

- Clop-M / Betnovate / Quadriderm / Panderm
- Luliconazole on eyelids

Prevention:

- Moisturize daily
- Gentle face wash only
- Sunscreen daily
- Avoid rubbing eyes
- Avoid hot showers

6. Healing Timeline

- Eyelid itching: 2–3 days
- Eyelid dryness: 2–3 weeks
- Body patches: 7–10 days
- Full skin barrier repair: 4–8 weeks